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No. 96-1793

IN THE
Supreme Court of the United States
OCTOBER TERM, 1997

CEDAR RAPIDS COMMUNITY SCHOOL DISTRICT,
Petitioner,

v.

GARRET F., A MINOR BY HIS MOTHER AND NEXT FRIEND, CHARLENE F.,
Respondent.

On Writ of Certiorari to the United States Court of Appeals for the Eighth Circuit

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BRIEF FOR THE PETITIONER

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QUESTIONS PRESENTED

1. Whether a school district is required to pay for continuous one-on-one nursing services for a disabled student, when the Individuals With Disabilities Education Act expressly excludes "medical services" from its mandate.

2. Whether there should be a bright-line rule that the Act's exclusion of "medical services" means only those services provided by a physician, or whether other medical services are excluded from the Act.

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BRIEF FOR THE PETITIONER

OPINIONS BELOW

The opinion of the Court of Appeals for the Eighth Circuit is reported at 106 F.3d 822 and is reproduced at Pet. App. 1a-7a. The opinion of the District Court for the Northern District of Iowa is unpublished, but appears at 24 Individ. Disab. Educ. L. Rep. 648 and is reproduced at Pet. App. 8a-16a. The unpublished decision of the Administrative Law Judge on behalf of the Iowa Department of Education appears at 22 Individ. Disab. Educ. L. Rep. 278 and is reproduced at Pet. App. 17a-63a.

JURISDICTION

The Court of Appeals for the Eighth Circuit entered its judgment on February 7, 1997. The Cedar Rapids Com-

munity School District filed a petition for writ of certiorari on May 8, 1997, which this Court granted on May 18, 1998. This Court has jurisdiction under 28 U.S.C. § 1254(1).

STATUTORY PROVISIONS INVOLVED

This case involves consideration of the Individuals With Disabilities Education Act (the "IDEA" or "Act"), 20 U.S.C. § 1400 *et seq.* The IDEA was amended in 1997 (1997 Amendments), Pub. L. No. 150-17. Unless otherwise noted, citations are to the 1994 codified version of IDEA in effect immediately prior to the 1997 Amendments, the statute in effect at the time Respondent sought funding for services.

20 U.S.C. § 1412 provides in pertinent part:

> In order to qualify for assistance under this subchapter in any fiscal year, a State shall demonstrate to the Secretary that the following conditions are met:
>
> (1) The State has in effect a policy that assures all children with disabilities the right to a free appropriate public education.

20 U.S.C. § 1401 provides in pertinent part:

> (a) As used in this chapter—
>
> * * * *
>
> (16) The term "special education" means specially designed instruction, at no cost to parents or guardians, to meet the unique needs of a child with a disability, including—
>
> (A) instruction conducted in the classroom, in the home, in hospitals and institutions, and in other settings; and
>
> (B) instruction in physical education.
>
> (17) The term "related services" means transportation, and such developmental, corrective, and other supportive services (including speech pathology and audiology, psychological services, physical and occupational therapy, recreation,

> including therapeutic recreation, social work services, counseling services, including rehabilitation counseling, and medical services, except that such medical services shall be for diagnostic and evaluation purposes only) as may be required to assist a child with a disability to benefit from special education, and includes the early identification and assessment of disabling conditions in children.
>
> (18) The term "free appropriate public education" means special education and related services that—
>
> (A) have been provided at public expense, under public supervision and direction, and without charge,
>
> (B) meet the standards of the State educational agency,
>
> (C) include an appropriate preschool, elementary, or secondary school education in the State involved, and
>
> (D) are provided in conformity with the individualized education program required under Sec. 1414(a)(5) of this title.

STATEMENT OF THE CASE

A. Garret F.'s Condition and Needs.

Respondent Garret F. is a ventilator-dependent child enrolled in the Cedar Rapids Community School District. In 1987, when Garret was four years old, he was seriously injured when his blanket caught in the drive mechanism of the motorcycle on which he was riding, jerking his head. Garret suffered a complete transection of the spine and, as a result, is quadriplegic and completely paralyzed below his neck. Garret depends on a ventilator for breathing. He has normal mental capacity, is able to speak, and uses a puff and sip straw to control a wheelchair. Pet. App. 19a-20a.

It is undisputed that Garret requires the following health services while at school:

- urinary bladder catheterization (usually once a day);
- suctioning of his tracheostomy as needed;
- change of positioning;
- ambu bag administration in cases of ventilator malfunctioning (this consists of manual pumping of an air bag attached to his tracheostomy tube);
- ventilator setting checks, observations and assessments to determine if Garret is in respiratory distress;
- assessment to determine if Garret is experiencing autonomic hyperreflexia; and
- blood pressure monitoring, urinary catheterization and bowel disimpaction in the event of an episode of autonomic hyperreflexia.[1]

Pet. App. 20a. These observation and monitoring services must be provided continuously by trained personnel because Garret could die if his breathing were interrupted for just a few minutes. Unlike every other student in the District who receives free health-related services from the District during the day, Garret F. requires "constant assessment and monitoring." TR at 338; EX 94 at 364-378.

B. Services Provided by the District.

For every year since Garret enrolled in the District in 1988, the District has provided him with an individualized educational plan ("IEP") written by a multi-disciplinary

[1] "Autonomic hyperreflexia" is a life-threatening abnormal response of the autonomic nervous system that can occur in a ventilator-dependent individual such as Garret. Among other things, it results in elevated blood pressure and must be treated immediately. Garret has experienced several episodes of autonomic hyperreflexia. TR at 76-77, 81-82.

educational team. Garret has always received instructional services in regular education classrooms.

The District has also hired an *educational* associate to assist Garret in moving about the building and in manipulating his District-provided computer and other instructional materials. The associate, who is compensated at the rate of approximately $9,500 a year, does not attend to Garret F.'s health-related needs. EX 128 at 463.[2]

Additionally, the District has provided Garret with special transportation; for the past several years, this has consisted of a special lift bus. Finally, Garret has been provided with specialized equipment paid for by the District, including a computer with a mouth stick and special software. TR at 217.

C. The Parties' Dispute Over the Obligation to Provide Continuous One-on-One Nursing Services for Garret F.

During the first several years of Garret's enrollment in the District, the District and Garret's parents entered into an agreement that the family would provide a full-time attendant for Garret while he was at school. EX 20 at 84-89; EX 26 at 96-115. The family's health insurance and a trust fund established after a settlement of legal claims arising out of the motorcycle accident covered the costs of this attendant. Since 1989, this attendant has been an L.P.N. who attends exclusively and continuously for Garret while he is at school. J.A. at 11.[3]

In 1993, when Garret entered fifth grade, Garret's mother, Charlene F., requested that the District bear the

[2] Although the District's educational associate in 1994 happened to be a licensed practical nurse ("L.P.N."), she was not called upon to provide nor compensated for providing any health-related services. The current educational associate is not an L.P.N.

[3] In addition, this L.P.N. and another L.P.N. provide services for Garret in his home. Garret receives one-on-one nursing services for 16 hours per weekday. J.A. at 12; TR at 129.

costs of the full-time attendant required by Garret. The District declined Charlene F.'s request that it pay for Garret's continuous nursing services, taking the position that the services in question were medical services which the District was not obligated to provide. EX 93 at 390. The District has been willing to contribute the full amount of the teaching associate's salary (approximately $9,500) toward the nurse's salary, with the understanding that the nurse would then fulfill the educational associate's duties as well.

The District—one of the largest in Iowa—operates approximately 33 schools for approximately 17,500 students. In 1994, approximately 2,200 of its students received special education services. The District has the equivalent of six full-time registered nurses to serve its 17,500 students and 33 schools. No building has a registered nurse continuously on site, and the District does not provide one-on-one continuous care for any other student. Extensive testimony appears in the record regarding duties of school nurses in Cedar Rapids. TR at 441-442, 477-80.[4] None of those duties includes continuous monitoring or treatment. The parties stipulated that, as of 1994, the District paid its least senior nurse approximately $37,000 including benefits, and that it would cost at least $27,000 to hire a special nurse for Garret. J.A. at 15-16.

The parties also stipulated that if the District employed the nurse, the costs of the teaching associate currently hired by the School District could be saved, as the District could assign the duties of the teacher associate to the nurse (helping Garret physically move about the building,

[4] These duties include planning health services for children; attending child study team meetings when children have health concerns; supervising and inservicing secretaries and staff who administer medications, give emergency first aid, and provide other intermittent care such as gastric feedings and CIC; providing health education in the classroom; insuring facility safety; checking student immunization records; and other record-keeping.

turn pages, manipulate materials). TR at 465. The additional costs to the District, as later calculated by the administrative law judge, were thus between $20,000 and $30,000. Pet. App. 53a.

Following the District's denial of continuous nursing services, Garret F.'s mother requested an administrative hearing pursuant to the Individuals With Disabilities Education Act (the "IDEA"). EX 96 at 401. The hearing was continued while the District sought additional information from the Iowa Board of Nursing. The Board of Nursing provided a declaratory ruling that the care required by Garret could not be delegated at school to a nonlicensed practitioner. EX 125 at 447-461. The Nursing Board rules also provide that care may not be delegated from a registered nurse (R.N.) to a licensed practical nurse (L.P.N.) unless an R.N. is in the same building at all times. EX 94 at 392-394.

Because the Iowa Board of Nursing had specifically ruled that the District could not delegate Garret's required care to anyone who was not a nurse, it was not possible to entrust the continuous monitoring of Garret to nonlicensed personnel, as Garret's physician Dr. Reinertson suggested. Also, the District nurse testified that even if the Board of Nursing had ruled that she could legally delegate the nursing services to a nonlicensed person, she would not have felt comfortable doing so. TR at 453-454.

D. The Administrative Hearing.

A state administrative hearing was held, and on December 16, 1994, an Iowa administrative law judge ruled that the District was required to provide for all of Garret's health needs at school including the continuous intensive nursing services he requires. Pet. App. 17a-63a. The ALJ read this Court's decision in *Irving Independent School Dist. v. Tatro*, 468 U.S. 883 (1984), as requiring the District to provide, under the IDEA, *any* health care

service that is not administered by a physician.[5] The ALJ stated, "As much as this ALJ may disagree with the public policy of spending education funds on health services, he does not feel he has the authority to change the law." Pet. App. 51a.

In dicta, the ALJ noted that even if the IDEA did not define "medical services" so narrowly as to include only services directly administered by a physician, and instead imposed a balancing test as several lower courts had found, the District still had to provide the requested one-on-one continuous services for Garret. Although the ALJ's opinion on this point is somewhat unclear, he appeared to suggest either that the District could hire a "conscientious lay person" (although the Iowa Nursing Board had ruled directly to the contrary), or that the financial burden of hiring a full-time R.N. was not so great as to require the service to be defined as "medical." Pet. App. 52a-53a. The ALJ also said that Iowa law required the District to pay for the nursing services as "corrective and supporting services." Pet. App. 54a.

E. The Federal Court Proceedings.

The District then filed an action in federal district court to review the ALJ's decision under 20 U.S.C. § 1415(e), requesting declaratory relief that "it is not required under either [the IDEA] or the Iowa Special Education laws" to provide the continuous one-on-one nursing services to Garret. On cross-motions for summary judgment, the district court granted summary judgment for Garret and his mother. The court ruled as a matter of law that the "medical services" exclusion in the Act "is limited to the services of a physician or hospital." Pet. App. 15a. The district court did not consider the nature or extent of the

[5] The ALJ had published an article the previous year which took the same position. Larry Bartlett, *"Economic Cost Factors in Providing a Free Appropriate Public Education for Handicapped Children: The Legal Perspective,"* 22 J.L. & Ed. 27, 55-56 (1993).

services or the burdens upon the District; nor did the district court consider the state law issues.[6]

The District appealed, and the Eighth Circuit Court of Appeals affirmed in an opinion adopting the same physician/non-physician distinction. The Eighth Circuit read this Court's decision in *Tatro* as "establish[ing] a bright-line test: the services of a physician (other than for diagnostic and evaluation purposes) are subject to the medical services exclusion, but services that can be provided in the school setting by a nurse or qualified layperson are not." Pet. App. 6a. The Eighth Circuit recognized that "several decisions . . . have not interpreted *Tatro* as establishing a bright-line, physician/non-physician test for medical services," and have instead factored into the analysis "considerations of the nature and extent of the services performed." Pet. App. 6a-7a (citing cases). The Eighth Circuit declined to follow those cases, concluding that it was bound by what it understood the holding in *Tatro* to be, "[r]egardless of whether we agree with this reading of the statute and the regulation." Pet. App. 6a. The District petitioned for certiorari to this Court. This Court granted the petition.

SUMMARY OF ARGUMENT

The Individuals with Disabilities Education Act (the "IDEA") does not require Cedar Rapids Community School District to hire a specially-trained nurse to attend continuously and exclusively to Garret F.'s health-care needs while he is at school. To require local school districts to provide intensive, continuous one-on-one nursing

[6] The district court's opinion incluudes a "But see" citation to *Neely v. Rutherford County School*, 68 F.3d 965 (6th Cir. 1995), one of the cases that used a practical, balancing approach to determine the scope of the "medical services" exclusion. Pet. App. 15a. Contrary to the suggestion of the Solicitor General at the petition stage, this confirms that the district court did *not* apply a balancing approach or find that Garrett F. would have prevailed under such an approach.

services as part of their "free and appropriate public education" mandate would undermine the literal language and the basic purpose of the federal law. The IDEA is an *education* law. Although it requires schools to provide certain "related services" as part of their mandate to provide special education, those services are enumerated, they do not include intensive nursing services, and they specifically exclude "medical services" except for evaluation and diagnosis. 20 U.S.C. § 1401(a)(17).

The term "medical" in the IDEA should have a plain, common-sense meaning, as it does when used in other federal statutes and in other contexts. "Medical" services do not have to be directly administered by a licensed physician, but can be furnished by other competent health care providers. Congress has considered and clarified the definition of "related services" several times when it amended the IDEA, and has made specific additions to the list of related services. However, Congress has never altered the original "medical services" exclusion, even though Congress was undoubtedly aware of several federal court precedents excluding one-on-one nursing services from the scope of the IDEA services.

Until this case, the pronouncements by the Department of Education also favored a common-sense definition of "medical services," not a formalistic definition turning on whether the person providing the service happened to be a licensed physician. As recently as 1996, two interpretive letters rejected a bright-line physician/nonphysician distinction and stated that private duty-type nursing services could be excluded from the coverage of the Act.

This Court's prior decisions in *Board of Education of Hendrick Hudson Central School Dist. v. Rowley*, 458 U.S. 176 (1982), and *Irving Independent School Dist. v. Tatro*, 468 U.S. 883 (1984), also lead to the conclusion that local school districts are not required to provide continuous one-on-one nursing services as part of their federal special education mandate. *Rowley* establishes two

key principles: first, that the IDEA was not intended to impose inflexible rules on states and localities; and, second, that the IDEA is "in the nature of a contract" between the federal government and the states, and that any requirements on states and localities must be spelled out "unambiguously." If Congress desired to have *only* physician services excluded from district responsibility, rather than all medical services, Congress could have clearly and easily stated this intent.

In *Tatro*, this Court held that school districts can be required to provide intermittent health-care related services, such as clean intermittent catheterization ("CIC"). CIC needed to be administered only once a day, could be administered by an existing school nurse, and was similar to services already provided by school nurses. In short, the same practical, flexible approach of *Tatro* that favored a finding that CIC is *included* in "related services" favors a finding of *exclusion* of continuous one-on-one nursing services here.

Ultimately, the question of whether health care services must be provided by a school district should not be decided by a mechanical physician/nonphysician test. Rather, the outcome should depend upon a series of factors, such as whether the care is continuous or intermittent, whether existing school health personnel can provide the service. the cost of the service, and the potential consequences if the service is not properly performed. Indeed, until the lower courts' decisions in this case, the vast majority of federal courts had followed just this approach. This multifactor test enhances the goal of the IDEA, which is to ensure that *all* disabled children receive a free appropriate public education by allocating scarce resources to education and education-related services, while utilizing other resources for medical services.

ARGUMENT

I. THE PLAIN LANGUAGE OF THE IDEA AND ITS LEGISLATIVE HISTORY CALL FOR ALL MEDICAL SERVICES, NOT JUST THOSE DIRECTLY PROVIDED BY A PHYSICIAN, TO BE EXCLUDED FROM THE SCHOOL DISTRICT'S RESPONSIBILITY.

The Individuals with Disabilities Education Act, 20 U.S.C. § 1400, *et seq.* (the "IDEA" or "Act")[7] serves the laudable goal of assuring that every disabled child will receive a free appropriate public education including "special education and related services" suitable to him or her. *Id.* § 1400(c). "Related services" are defined as "transportation, and such developmental, corrective, and other supportive services . . . as may be required to assist a child with a disability to benefit from special education. . . ." *Id.* § 1401(a)(17). The Act requires that these services be free of charge to the student. In return, the federal government provides assistance to the States, currently paying approximately seven percent of the costs of educating disabled children, leaving the States and local school districts to shoulder the rest of the costs. Chambers, Parrish, Lieberman & Wolman, "*What Are We Spending on Special Education in the U.S.?*" Center for Special Education Finance (funded by Office for Special Education Programs, Department of Education), Brief No. 8, February 1998.

However, consistent with the notion that the IDEA is about education, and is not a broader law requiring school districts to meet every daytime life-sustaining need of every student within their boundaries, the Act expressly excludes "medical services" from the "related services"

[7] When first enacted in 1975, the Act was known as the Education for All Handicapped Children Act. See Pub. L. 94-142, 84 Stat. 175. For convenience, this brief will refer to the Act as the Individuals with Disabilities Education Act or the "IDEA," the name it has had since 1990.

which must be provided by school districts. 20 U.S.C. § 1401(a)(17). School districts only have to provide and pay for medical services "for diagnostic and evaluation purposes." *Id.*

Garret, a 15-year-old ventilator-dependent child, requires the continuous assistance of medically trained personnel to attend to his medical needs. Without this assistance, Garret could suffer respiratory distress and be in a life-threatening medical condition. By any common-sense definition of "medical services," the intensive, one-on-one full-time nursing care that Garret needs to stay alive is a "medical" service. These same nursing services needed at both home and at school have been paid for by private health insurance as "medical services." J.A. at 12; TR at 129. Nonetheless, the District has been ordered to pay for these services at school because the courts below concluded that "medical services" means only those services "directly administered by a physician." Pet. App. 6a-7a.

Neither the express language of the Act, the legislative history of the Act, the Department of Education's regulations or interpretations, this Court's prior decisions, the majority of other courts' decisions, nor any other reliable source of statutory interpretation supports this unduly narrow definition of excluded medical services. To the contrary, the only logical interpretation of the Act favors a practical, common-sense application of the "medical services" exclusion so as to exempt school districts from having to pay for continuous one-on-one nursing services.

A. The Express Language of the IDEA Favors a Common-Sense Definition of Medical Services.

In interpreting a statute, the cardinal rule is first to examine the plain and ordinary meaning of the words of the statute. *Connecticut Nat'l Bank v. Germain*, 503 U.S. 249, 253-254 (1992); *United States v. Ron Pair Enters.*, 489 U.S. 235, 242 (1989). The language in the IDEA is clear: it expressly excludes *all* "medical services," except

those that are for diagnostic and evaluation purposes. As this Court has frequently reiterated, "[i]n the absence of an indication to the contrary, words in a statute are assumed to bear their 'ordinary, contemporary, common meaning.'" *Walters v. Metropolitan Educational Enterprises*, 117 S. Ct. 660, 664 (1997) (quoting *Pioneer Investment Services Co. v. Brunswick Associates Ltd. Partnership*, 507 U.S. 380, 388 (1993)). The term "medical services" is simply not understood in common parlance to mean only those services provided by a physician, but is instead understood to embrace a broad range of services related to diagnosing, curing, mitigating, or preventing a disease or other health-related condition. The phrase typically has this same meaning when used in other legislation. *See, e.g.*, 26 U.S.C. § 213(d) (definition of "medical care" in the Internal Revenue Code). Had Congress intended to exclude only those medical services directly administered by physicians, it could have simply said "except physician services" or "except doctor's services."

Congress was well aware of what "medical" meant when it adopted the Act in 1975. In just the previous decade, Congress had enacted sweeping Medicaid legislation as Title XIX of the Social Security Act which expressly defined "medical assistance" as including an ambit of medical services going well beyond those administered directly by licensed physicians. 42 U.S.C. § 1396d(a). The Internal Revenue Code had long defined "medical care" as amounts paid "for the diagnosis, cure, mitigation, treatment, or prevention of disease, or for the purpose of affecting the structure or function of the body. . . ." 26 U.S.C. § 213(d)(1). No one has ever seriously contended that the Tax Code's medical expense deduction is limited to amounts paid to physicians. The Federal Food, Drug and Cosmetic Act, 21 U.S.C. § 301, *et seq.*, employs a similar definition for determining what drugs and "medical devices" are subject to the regulation of the Act. *See* 21 U.S.C. § 321.

Likewise, there is a large body of precedent in other contexts holding that "medical" services are not only rendered by physicians. *See, e.g., Moore v. Metropolitan Life Ins. Co.*, 307 N.E.2d 554 (N.Y. 1973) ("medical services . . . obviously may be performed by any of a number of professionals or paramedical personnel"); *Warren Tucking Co. v. Chandler*, 277 S.E.2d 488, 492 (Va. 1981) (ordinarily nursing services are among the "medical" benefits that employers and workmen's compensation carriers must provide); *State ex rel. Multimedica, Inc. v. Snowden*, 647 N.E.2d 1374, 1378 (Ohio 1995) ("medical" refers to the science and art of the investigation, prevention, cure and alleviation of disease); *State v. Robinson*, 735 P.2d 801, 809 n.9 (Ariz. 1987) ("medical" providers include psychologists).

B. Congressional Findings and the Structure of the IDEA Emphasize that Schools Are to Utilize Their Resources for Educational Services While Other Providers Are Responsible for Medical Services.

The Congressional findings in the IDEA emphasize that the Act is intended to provide assistance to States in making provision for *educational* services:

> The Congress finds that—
>
> (3) more than half of the children with disabilities in the United States do not receive appropriate *educational* services which would enable them to have full equality of opportunity. [20 U.S.C. § 1400 (b)(3) (emphasis added).]

The Congress reinforced and strengthened the emphasis on educational services in 1997 when it reauthorized and amended the Act:

> The Congress finds the following:
>
> (1) . . . Improving *educational* results for children with disabilities is an essential element of our national policy . . .

(2) Before the date of the enactment of the Education for All Handicapped Children Act of 1975 (Public Law 94-142) . . .

(3) more than one-half of the children with disabilities in the United States did not receive appropriate *educational* services that would enable such children to have full equality of opportunity.

(5) Over 20 years of research and experience has demonstrated that the *education* of children with disabilities can be made more effective by—

. . .

(G) Focusing resources on teaching and learning while reducing paperwork and requirements that do not assist in improving *educational* results.

(6) While States, local educational agencies and educational service agencies are responsible for providing an *education* for all children with disabilities it is in the national interest that the Federal Government have a role in assisting State and local efforts to *educate* children with disabilities in order to improve results for such children and to ensure equal protection of the law. [IDEA Amendments of 1997, Pub. L. 105-17, Title I § 601, 111 Stat. 37, to be codified at 20 U.S.C. § 1400(c) (emphasis added).]

Consistent with the theme that school districts are to devote their resources to *education*, the Act also requires educators to *coordinate* with other government agencies in providing noneducational services that children may need. While the Act holds schools responsible for providing a free appropriate public education, including special education instructional and related services, see 20 U.S.C. §§ 1412(2)(B) & 1401(a)(18), it also specifically states that the Act does not relieve any other agency of its obligations to provide other services, or indeed to pay for certain related services (such as physical therapy and counseling). 20 U.S.C. § 1412(6). In fact, the Act requires states to enter into interagency agreements that define the financial responsibilities of the various agencies.

20 U.S.C. § 1413(a)(13). This language was further strengthened by the 1997 Amendments. Pub. L. 105-17, Title I § 612, 111 Stat. 60 to be codified at 20 U.S.C. §§ 1412(a)(11)(B) & 1412(a)(12).

Moreover, Congress specifically used the same term—"medical" services—elsewhere in the Act, to emphasize that other agencies remain liable for such costs:

> Nothing in this title permits a State to reduce *medical* and other assistance available, or to alter eligibility, under Titles V and XIX of the Social Security Act with respect to the provision of a free appropriate public education for children with disabilities in the State. [Pub. L. 105-17, Title I, Sec. 612, 111 Stat. 60, to be codified at 20 U.S.C. § 1412(e) (emphasis added)]

It would be illogical for Congress to state in one section of the law that other agencies retain responsibility for "medical" services which are broadly defined in Titles V and XIX, and then to give a narrow interpretation to excluded "medical" services in another section of the same law that would result in school districts assuming this responsibility.

Congress clearly originally intended, and has since reemphasized the intent, that schools are to utilize their resources for educational purposes and are not required to undertake financial responsibility for many other services, even if necessary for a child to benefit from the educational services offered by the District. This philosophy is present in the definition of "related services," where medical services are excluded except for diagnostic and evaluation purposes. Congress thus *explicitly* rejected the concept that "medical services" should be paid for by school districts, even though such services may be required to assist a child with a disability in receiving an education. 20 U.S.C. § 1401(a)(17).

C. Legislative Amendments to the IDEA Demonstrate that Congress Intended a Common-Sense Interpretation of Excluded Medical Services.

When Congress has been dissatisfied with the scope of free services being provided by school districts to disabled students under IDEA, it has not hesitated to say so explicitly. For example, in 1990, Congress expressly expanded the definition of required "related services" to include "social work services" and "rehabilitation counseling," but did nothing to affect the existing statutory definition of "medical services." Congress added the "social work services" and "rehabilitation counseling" provisions because, according to the House Committee Report, it had received reports that these services were not uniformly provided, even though the regulations or prior committee reports had directed that local school districts provide them. H. Rep. No. 101-544 at 7 (1990). Accordingly, Congress felt it was necessary to "clarify" this aspect of the IDEA. *Id.*

By contrast, Congress did not narrow or "clarify" the "medical services" exclusion in 1990 even though there was a published and well-publicized opinion holding one-on-one nursing services to be excluded medical services. *Detsel v. Board of Educ. Of Auburn Enlarged City School Dist.*, 820 F.2d 587 (2nd Cir.) (per curiam), *cert. denied*, 484 U.S. 981 (1987). Another district court had also exempted a school district from having to provide continuous one-on-one nursing services. *Bevin H. v. Wright*, 666 F. Supp. 71 (W.D. Pa. 1987).

Indeed, the 1990 amendments to the IDEA confirm that Congress did not want the Act to become more than an education law. In adding a definition of "transition services" (to assist disabled students about to leave school), the House Committee emphasized that schools should not be "asked to do what they are not intended to do." H. Rep. No. 101-544 at 11 (1990).

Nor did Congress, as recently as June of 1997, amend or "clarify" the term "medical services" when it passed the reauthorized IDEA, even though it was aware that most lower courts, including three circuit courts, had interpreted the term as excluding one-on-one nursing services. Pub. L. 105-17, Title I § 602, 111 Stat. 42, to be codified at 20 U.S.C. § 1401(22). Rather, the emphasis is even stronger now upon providing necessary *educational* services, so that disabled children have an opportunity to attend school, while at the same time sparing school districts from having to undertake non-educational expenses.

D. The Costs of Providing Continuous One-On-One Nursing Services to Garret Are Significantly Beyond the Educational Resources the District Is Required to Expend.

Congress has also recognized that the federal financial contribution to the education of disabled students is limited, and therefore the burdens imposed on local school districts must have some limits as well. In 1994, the federal government's share of funding for special education services was only $413 for each child served. *Eighteenth Annual Report to Congress on the Implementation of the IDEA*, U.S. Department of Education (1996). In Iowa, none of this funding goes directly to local school districts.

At the time of the 1994 administrative hearing in this case, the District received approximately $12,780 in state and local income and property taxes on account of Garret's enrollment, an amount that is supposed to provide sufficient resources for educational and related services for someone of Garret's level of disability. TR at 513-514. The District expends funds not only for the regular educational and activity programs for Garret, but also for his educational associate, his special transportation, and his computer equipment. Pet. App. 34a-36a. Thus, even without taking into account Garret's need for continuous nursing services, the funding for his needs is inadequate.

Hiring a nurse to care for Garret from the time he left home on the bus until he returned home would have cost between $28,630 and $39,810. J.A. at 15. There was no dispute in the record that nurses hired in Cedar Rapids are registered nurses carrying a bachelor of science degree. J.A. at 17. The variation in estimates was based upon whether a nurse with no experience could be hired or a typical nurse employed in the District with ten years experience. This cost estimate was consistent with cost estimates of one-on-one nursing services in other cases where courts have declined to impose the costs on the school district. In *Ellison v. Board of Educ. of Three Villages Central School Dist.*, 597 N.Y.S. 2d 483 (App. Div. 1993), it was estimated that the cost of one-on-one nursing would be $25,000 to $40,000. In *Granite School Dist. v. Shannon M.*, 787 F. Supp. 1020 (D. Utah 1992), it was estimated to be approximately $30,000, and in *Fulginiti v. Roxbury Tp. Public Schools*, 921 F. Supp. 1320 (D.N.J. 1996), *aff'd, without published opinion*, 116 F.3d 468 (3rd Cir. 1997), the costs were estimated to be $56,000.

If Congress had intended that school districts were to provide for costly private nursing services, such as Garret requires, it would have specifically stated so in its definition of "related services," just as it did for other potentially expensive services, such as physical and occupational therapy. Instead, Congress chose to specifically exclude "medical services" unless for diagnostic or evaluation purposes. If public school entities are to use their limited resources to fund such expensive individualized nursing services, then that should be required by specific Congressional action rather than by a narrow interpretation of the term "medical." This Court has repeatedly stated that if Congress desires to condition states' receipt of federal funds, it must do so unambiguously. *Pennhurst State School and Hospital v. Halderman*, 451 U.S. 1, 17 (1981); *South Dakota v. Dole*, 483 U.S. 203, 207 (1987); *New York v. United States*, 505 U.S. 144, 172 (1992). *See*

Gebser v. Lago Vista Independent School Dist., S. Ct. No. 96-1866 (June 22, 1998), slip op. at 10-12 (Title IX scheme, imposing conditions on receipt of federal funds, requires careful scrutiny to ensure recipients had clear notice of nature and scope of conditions). *Accord Commonwealth of Virginia, Dept. of Ed. v. Riley*, 106 F.3d 559, 560-61 (4th Cir. 1997) (applying this principle to the IDEA). Indeed, in *Board of Education of Hendrick Hudson Central School Dist. v. Rowley*, 458 U.S. 176, 190 n.11 (1982), discussed below, this Court described this proposition as "fundamental." This proposition should be especially true when, as here, Congress is paying only about seven percent of the costs of the program.

II. THE DEPARTMENT OF EDUCATION'S REGULATIONS AND INTERPRETATIONS DO NOT SUPPORT AN UNDULY RESTRICTIVE DEFINITION OF THE MEDICAL SERVICES EXCLUSION.

The Department of Education regulations implementing the IDEA do not address the question at issue in this case: what medical services are excluded from a school district's responsibility under 20 U.S.C. § 1401(a)(17). The regulations *do* define covered medical services for which a district *is* responsible, *see* 34 C.F.R. § 300.16(b)(4), but that is a different question. There is no basis for inferring that it is the Department's considered view that any type of service that does not fit within its definition of a *covered* medical service is not a medical service at all, and therefore not subject to the medical services exclusion. In fact, as detailed below, the available evidence confirms that such an inference is not the Department's position.

The Department's regulations, which date back to 1977 with relatively minor modification, define the term "related services" as follows:

> The term "related services" means transportation and such developmental, corrective, and other supportive services as are required to assist a child with a dis-

> ability to benefit from special education, and includes speech pathology and audiology, psychological services, physical and occupational therapy, recreation, including therapeutic recreation, early identification and assessment of disabilities in children, counseling services, including rehabilitation counseling, and medical services for diagnostic or evaluation purposes. The term also includes school health services, social work services in schools, and parent counseling and training. [34 C.F.R. § 300.16(a)]

"School health services" are separately defined in § 300.16(b)(11) as "services provided by a qualified school nurse or other qualified person." Lastly, 34 C.F.R. § 300.16(b)(4) defines *covered* "medical services" as "services provided by a licensed physician to determine a child's medically related disability that results in the child's need for special education and related services." There is no definition of excluded medical services.[8]

Thus, the regulations leave the critical question in this case—the definition of *excluded* medical services—unanswered. As the Solicitor General acknowledged in the brief filed at the petition stage, "the regulations expressly define only those medical services that *are* owed to handicapped children. . . ." SG Br. 3 n.2 (emphasis in original). Simply because the Department of Education has concluded that the covered medical services of diagnosis and evaluation are to be provided by a physician does not mean that the only excluded medical services are those similarly provided by a physician. It may make sense, in considering the covered services of diagnosis and evaluation, to conclude that such services should be provided by

[8] The Department of Education has published proposed rules to implement the 1997 Amendments to IDEA. 62 Fed. Reg. 204 (October 22, 1997). The proposed regulations do not alter the provisions relevant to this case, but merely replace the term "speech pathology and audiology" with "speech language pathology and audiology services" and add "orientation and mobility services," in order to conform to statutory changes.

a physician. But that hardly suggests that there can be no such thing as a medical service that is not provided directly by a physician.[9]

The definition of "school health services" in the regulations as "services provided by a qualified school nurse or other qualified person" is of no help to Respondent. Since the regulation clearly does not mean that *all* requested services must be provided by the district, it is most reasonably read as providing that the term "school health services" encompasses services *analogous to* those traditionally provided by school nurses. In other words, school districts do not have to provide services that significantly differ in nature and magnitude from traditional "school health services." As discussed below, this Court in *Tatro* took that approach.

Thus, nothing in the regulatory definition of "school health services" suggests that the intensive, one-on-one care required by Garret F. is the responsibility of the School District, and nothing in the regulatory definition of covered medical services suggests that this care is not within the medical services exclusion.

In any event, the Department of Education has not previously taken the position that its regulations require school districts to bear the expense of every student-related medical treatment that was not directly administered by a physician. Indeed, in 1981, the Secretary of Education expressly acknowledged that the Department's regulations in this area were "ambiguous." In postponing an interpretive ruling he had planned to deliver on clean inter-

[9] In *Irving Independent School Dist. v. Tatro*, 468 U.S. 883, 893 (1984), this Court stated that the regulation "presumably . . . means that 'medical services' *not* owed under the statute are those 'services by a licensed physician' that serve other purposes" (emphasis in original). The Court cited no authority for its presumption that "medical" really means "physician," and this was not critical to the *Tatro* finding.

mittent catheterization ("CIC") (the service involved in *Tatro*), the Secretary stated:

> The Secretary is also concerned that the differentiation between related services and medical services be clarified. The Secretary declines to issue an interpretation based on regulations that are *ambiguous* and subject to review. [46 Fed. Reg. 25614 (1981) (emphasis added)]

The following year, the Secretary proposed additional regulations in six general areas of the IDEA, including the related services/medical services distinction. The Secretary's notice of proposed rulemaking reiterated the perceived ambiguities in the original regulations::

> In recent years, school officials and parents have pointed out problems in the implementation of the current statutory and regulatory requirements concerning related services. *The most persistent problems involve determining whether certain health-related services are to be provided.* [47 Fed. Reg. at 33837 (1982) (emphasis added)]

Among other things, the Secretary's 1982 proposed rules would have expressly exempted school districts from providing "life-sustaining procedures that . . . must be administered by specially-trained, licensed health care professionals. . . ." *Id.* at 33846. In short, the 1982 proposed rules would even have excluded procedures that could be done on an intermittent basis by school nurses, such as clean intermittent catheterization. As the government correctly noted in its brief at the petition stage, the Secretary's wide-ranging notice of rulemaking provoked opposition in Congress and was later withdrawn. The Secretary announced plans to issue a new notice of proposed rulemaking, but did not do so. 47 Fed. Reg. 49871 (1982). In 1983, Congress then added a largely symbolic provision to the IDEA prohibiting the Secretary from implementing any regulation which:

> would procedurally or substantively lessen the protections provided to children with [disabilities] under this chapter, as embodied in regulations in effect on July 20, 1983 (particularly as such protections relate to parental consent to initial evaluation or initial placement in special education, least restrictive environment, related services, timeliness, attendance of evaluation personnel at individualized educational program meetings, or qualifications of personnel), except to the extent that such regulation reflects the clear and unequivocal intent of the Congress in legislation. [20 U.S.C. § 1407(b)]

Yet nothing in this debate from the early 1980's suggests that either the Department of Education or Congress envisioned a rule limiting the medical services exclusion to physician services. At the time, the greater debate about medical services revolved around individualized services which a school nurse could provide to disabled children while performing other normal duties for several hundred students. (*Tatro* was decided the following year.) None of the debate centered on a discussion of one-on-one private-duty nursing. The foregoing thus hardly supports the Solicitor General's extravagant assertion in his brief at the petition stage that Congress has somehow endorsed a non-existent regulatory position that such care was the responsibility of a school district under the Act.

The United States Department of Education has issued a number of interpretive letter rulings regarding the issue of what constitutes a medical service under the Act, including two letter rulings on nursing services.[10] These interpretations have not applied a bright-line distinction

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[10] See *Hehir to Anonymous*, 25 Individ. Disab. Educ. L. Rep. 531 (November 13, 1996); *Hehir to Anderson*, 24 Individ. Disab. Educ. L. Rep. 180 (February 22, 1996); *Guard to Del Polito*, Educ. Handicap L. Rep. 211:392 (June 24, 1986); *Irvin to Millman*, Educ. Handicap L. Rep. 211:104 (May 22, 1979); *Tyrrell to Janda*, Educ. Handicap L. Rep. 211:205 (January 25, 1979); *Irvin to Minsky*, Educ. Handicap L. Rep. 211:19 (April 7, 1978).

of whether the services are performed by physicians or nonphysicians.

Most recently, the agency clearly renounced any "bright-line" physician/nonphysician test. *Hehir to Anderson*, 24 Individ. Disab. Educ. L. Rep. 180 (February 22, 1996), Pet. App. 64a-67a. The Director of the Office for Special Education Programs ("OSEP") was specifically asked for the first time whether one-to-one nursing services necessary for a student to attend a public school setting were the responsibility of a school district. The agency specifically rejected the "bright-line" analysis and instead said that each case had to be determined on a case-by-case basis in light of the particular facts and circumstances involved. For this reason, OSEP refused to express any "view as to whether or not 'one-to-one nursing' services are a required related service for an individual disabled student." Pet. App. 67a. The agency carefully made a distinction between whether the required care was *intermittent* and could be provided by a *regular* school nurse or whether the care was continuous and was more in the nature of private duty nursing:

> A threshold issue raised by your inquiry is whether "one-to-one nursing services" to enable a disabled student to attend school would be considered an eligible "school health service," or an excluded "medical service" because the services are for purposes other than those that are "diagnostic and evaluative in nature." 34 CFR § 300.16(b)(4) and (b)(11). Courts that have addressed this issue have reached differing conclusions based on factors such as the nature and intensity of the nursing services required for a particular student and the level of skill required of the individual performing the services. If the care required is intermittent and could be provided by a regular school nurse, courts have generally held that the service is an eligible related service. *See e.g., Irving Independent School District v. Tatro*, 104 S. Ct. 3371 (1984), *Department of Education, State of Hawaii v. Katherine D*, 727 F.2d 809 (9th Cir. 1984), *cert. den.* 471 U.S. 1117 (1985) and *Macomb County Intermediate School District v. Joshua S*, 715 F. Supp. 824 (E.D. MI. 1989). However, if the care required is continuous, courts relying on the private duty aspect of the services, generally have held that the service is an excluded medical service. *See e.g., Neely v. Rutherford County School District*, 68 F.3d 965 (6th Cir. 1995); *Detsel v. Board of Education of Auburn Enlarged City District*, 637 F. Supp. 1022 (N.D. N.Y. 1986) *Aff'd per curiam*, 820 F.2d 587 (2d Cir. 1987), *cert. den.* 484 U.S. 981 (1987); *Granite School District v. Shannon M.*, 787 F. Supp. 1020; (D. Utah 1992) and *Bevin H. v. Wright*, 666 F. Supp. 71 (W.D. Pa. 1987).

Later in 1996, the Department of Education again made the distinction between intermittent care which could be provided by a regular school nurse and continuous care which was more in the nature of private duty nursing in another letter ruling. *Hehir to Anonymous*, 25 Individ. Disab. Educ. L. Rep. 531 (November 13, 1996).

Therefore, since the original passage of the IDEA in 1975, and continuing to this date, there has been no regulatory definition of excluded "medical services," and there has been only a general definition of required "school health services." At the same time, the interpretations by the agency itself have rejected a bright-line rule in favor of a case-by-case analysis.

III. THIS COURT'S PRIOR DECISIONS REQUIRE THAT THE IDEA, INCLUDING THE MEDICAL SERVICES EXCLUSION, BE INTERPRETED IN A PRACTICAL, COMMON-SENSE MANNER, AN APPROACH FOLLOWED IN MOST SUBSEQUENT LOWER COURT DECISIONS.

A. This Court's *Rowley* Decision.

This Court's decision in *Board of Education of Hendrick Hudson Central School Dist. v. Rowley*, 458 U.S. 176 (1982), provides the starting-point for examining judicial

interpretations of the IDEA. In *Rowley*, this Court's first decision under the IDEA, this Court considered whether school districts had to provide every service that was needed to give a disabled child an opportunity "commensurate with the opportunity provided other children." *Id.* at 198 (quoting the district court). The parents in *Rowley* sought to require the school district to provide a sign-language interpreter for their partially deaf child, arguing an interpreter was needed to equalize their daughter's opportunities with those of nondisabled children even though she was performing well in school. *Id.* at 185-86.

The Court rejected the parents' argument, holding that the Act did not require any particular level of educational benefits. *Id.* at 202-203. The Court expressly rejected the notion that school districts were obligated by the Act to "maximize each child's potential," regardless of burden or cost. *Id.* at 204 n.26. Instead, the facts in each case had to be weighed individually. *Id.* at 202-203. Accordingly, it reversed the lower court rulings that required Amy Rowley to be provided with a sign language interpreter while attending school.

Rowley repeatedly emphasizes Congress's desire to afford states flexibility in meeting the requirements of the IDEA. Congress did not impose "a burden of unspecified proportions and weight" when it enacted the IDEA. 458 U.S. at 190 n.11. The IDEA does not "guarantee any particular level of education," and imposes no clear obligation upon recipient States beyond the requirement that handicapped children receive some form of specialized education. . . ." *Id.* at 192, 195. One cannot square these kinds of statements, and *Rowley*'s rejection of bright-line rules, with a bright-line rule that school districts provide nursing services regardless of burden or cost.

B. This Court's *Tatro* Decision.

Practical considerations also guided this Court in *Irving Independent School Dist. v. Tatro*, 468 U.S. 883 (1984), its only prior decision addressing the "related

services" and "medical services" definitions. In *Tatro*, this Court generally adopted a flexible approach in ruling for a school-age child with spina bifida who needed clean intermittent catheterization ("CIC") every three or four hours. The CIC procedure, according to the Court, was "a simple one that may be performed in a few minutes by a layperson with less than an hour's training." *Id.* at 885. Indeed, the Court noted that Amber Tatro would soon be able to perform the procedure herself. It was also undisputed that state law did not require even a nurse to perform this simple procedure. *Id.* at 887. Under these circumstances, this Court held that the school district should provide the service. This Court's reasoning had two components:

(1) CIC is analogous to the type of services that school nurses have traditionally provided to nondisabled students, such as dispensing oral medications and giving emergency injections. In other words, CIC "services [are] of a kind that are routinely provided to the non-handicapped." *Id.* at 893-894.

(2) While one could conclude that Congress wanted "to spare schools from an obligation to provide a service that might well prove unduly expensive and beyond the range of their competence," it was also logical to conclude that "Congress intended to impose the obligation [on schools] to provide school nursing services." *Id.* at 892-893.

In short, this Court found that CIC, a simple procedure that needed to be performed only once during the school day by a school nurse who was not thereby precluded from performing services for other children, was a "related" service analogous to traditional school nursing services (like administering insulin shots for a diabetic child) and not an excluded "medical service." [11]

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[11] The Cedar Rapids Community School District currently provides intermittent catheterization for approximately five students. TR at 287. By way of contrast, the District does not provide continuous nursing services for any student.

As in *Rowley*, this Court in *Tatro* largely followed a balanced, nuanced approach, carefully considering the burdens on the school district. *Id.* at 893-894. Some language in *Tatro* does support a bright-line physician/nonphysician rule, such as the Court's statement that the Secretary of Education "limit[ed] the 'medical services' exclusion to the services of a physician or hospital," *Id.* at 893. *See* Haekyoung Suh, Note, "*The Need for Consistency in Interpreting the Related Services Provision Under the Individuals With Disabilities Education Act*," 48 Rutgers L.J. 1321, 1325-26 (1996) (discussing the two sides to *Tatro*). Yet even here, the Court noted in the same sentence that physician services are "far more expensive" than school nursing services. *Id.* at 893. That was true when considering the time of a school nurse to administer a simple procedure once a day. Continuous, one-to-one nursing services of the sort at issue here are in fact much more expensive than an office visit to a physician.

Significantly, in *Tatro* the parents, not the school district, were advocating a flexible approach. The school district argued for a formalistic interpretation of the IDEA that characterized CIC as a medical service because it had to be prescribed by a physician, even though it only needed to be administered once a day. The parents argued for a more practical approach, contending that CIC was not a genuine medical service because, among other circumstances, Amber Tatro would soon be able to administer it to herself.

C. Lower Court Decisions on the "Medical Services" Exclusion, Until This Case, Have Almost Without Exception Favored a Practical Approach.

Until the decisions in this case, almost all lower courts, as well as state administrative hearing officers, had read *Tatro* as endorsing a pragmatic and balanced definition of "medical services." Indeed, at least five previously reported federal court decisions concluded that continuous

one-on-one nursing services were an excluded "medical service" under the IDEA. This suggests that this opinion in *Tatro* clearly favored such an approach, and that such an approach makes sense given the intensive nature of the services involved.

In *Clovis Unified School Dist. v. California Office of Admin. Hearings*, 903 F.2d 635, 644 (9th Cir. 1990), the Ninth Circuit addressed whether a child's residential placement was an excluded "medical service" and concluded that it was. As the court stated:

> [*Tatro*] held only that services which *must* be provided by a licensed physician, other than those which are diagnostic or evaluative, *are* excluded and that school nursing services of a simple nature are not excluded. In reaching this decision the Court considered the extent and nature of the services performed, not solely the status of the person performing the services.

The Ninth Circuit added, "[I]t would do havoc to the structure of the Act to exclude only the services of licensed physicians . . ., and to require the school district to pay for all other services." *Id.* The court also rejected a rule that would require school districts to pay for all services that are "*supportive* of a handicapped child's education." *Id.* at 643 (emphasis added). The court noted that this test was "far too inclusive . . . in light of the Act's explicit exclusion of medical services," because many medical treatments are "supportive" of education. *Id.*

The Sixth Circuit read *Tatro* similarly in *Neely v. Rutherford County School*, 68 F.3d 965 (6th Cir. 1995), *cert. denied*, 517 U.S. 1134 (1996), a case involving continuous one-on-one nursing services. There the court held that the tracheostomy suctioning and potential ambu bagging services required by Samantha Neely were excluded "medical services." Samantha had a rare condition which required her to breathe through a tracheostomy tube. The tube had to be suctioned and, if her breathing

stopped, artificial ventilation with an ambu bag would have to be provided. Given that a nurse or medical attendant "would have had to devote virtually all of his or her attention" to Samantha, and that "such care was necessary in order to protect" the child's life, the court held that the services her parents sought were "medical" and not "related." *Id.* at 971-973.

In discussing this Court's *Tatro* decision, the Sixth Circuit noted that it was "subject to several interpretations." *Id.* at 970. The Sixth Circuit concluded that "the better interpretation of *Tatro*" is that a school district is not required to provide every service that is "medical in nature," but rather that the burden on the school district must be considered along with the nature and extent of the services. *Id.* at 971.

Likewise, in *Detsel v. Board of Educ. of the Auburn Enlarged City School Dist.*, 820 F.2d 587 (2nd Cir. 1987) (per curiam), *aff'g* 637 F. Supp. 1022 (N.D.N.Y. 1986), *cert. denied*, 484 U.S. 981 (1987), another continuous nursing services case, the Second Circuit held that the daily nursing service for a child who needed constant respirator assistance was an excluded "medical service" and not a covered "related service" within the Act. The court approved the district court's opinion which "gave proper effect to the statuory scheme in balancing the interests of the parties." *Id.* at 588. The district court had found that the "extensive, therapeutic health services sought by the plaintiff on behalf of her daughter more closely resemble the medical services specifically excluded by § 1401(17) of the [IDEA]." 637 F. Supp. at 1027. As the district court explained:

> [T]he *Tatro* decision does not require the provision of all health services, regardless of their magnitude, if performed by one other than a physician. The Supreme Court held only that school nursing services of a simple nature are not excludable as therapeutic "medical services." [*Id.*]

Most recently, in *Fulginiti v. Roxbury Tp. Public Schools*, 921 F. Supp. 1320 (D.N.J. 1996), *aff'd without published opinion*, 116 F.3d 468 (3rd Cir. 1997), the district court, affirmed by the Third Circuit, held that ongoing nursing services for a child with dysfunction of her nervous system were an uncovered "medical service." Carissa Fulginiti had a tracheostomy tube that required monitoring and suctioning during the school day. The court found that "the care required for Carissa is medical in nature and that to provide it would be unduly burdensome upon the District." *Id.* at 1325.

These court of appeals decisions have been joined by federal district court decisions that read *Tatro* similarly. In *Granite School Dist. v. Shannon M.*, 787 F. Supp. 1020 (D. Utah 1992), the district court ruled that full-time nursing care for a child with neuromuscular atrophy and severe scoliosis was not a "related service" under the IDEA which the school district had to provide as part of a free and appropriate public education. The student used a tracheostomy tube that required constant attention; the court reasoned that this constant care could not be provided by the school district's three existing nurses, and that "the Act does not require Granite to provide Shannon with full-time nursing/tracheostomy care as a supportive service." *Id.* at 1030.

In its review of the *Tatro* decision, the court noted:

> The specific holding of *Tatro* is that CIC service is a supportive service not subject to the medical services exclusion of the Act. The court does not read *Tatro* to stand for the proposition that *all* health services performed by someone other than a licensed physician are related services under the Act regardless of the amount of care, expense, or burden on the school system and, ultimately, on other school children. [*Id.* at 1026.]

Accord Bevin H. v. Wright, 666 F. Supp. 71, 74-76 (W.D. Pa. 1987) ("related services" do not include one-

on-one nursing to care for a disabled student with a tracheostomy).

Finally, in *Ellison v. Board of Educ. of Three Villages Central School Dist.*, 597 N.Y.S.2d 483 (App. Div. 1993), a New York appellate court considered a claim brought on behalf of a child who, like Garret, was quadriplegic and ventilator-dependent. The court held that the one-on-one nursing services comparable to those required by Garret were not "simple school nursing services" but rather excluded medical services.

Until this case reached the federal courts, only one post-*Tatro* reported decision had found that "medical services" *were* limited to services directly administered by physicians. *Macomb County Intermediate School Dist. v. Joshua S.*, 715 F. Supp. 824, 828 (E.D. Mich. 1989). Since the Eighth Circuit found for Garret F., two other district courts have adopted this position, while acknowledging that their position is in the minority. *Morton Community Unit School Dist. No. 709 v. J.M.*, 986 F. Supp. 1112, 1119 (C.D. Ill. 1997); *Skelly v. Brookfield LaGrange Park School Dist.*, 968 F. Supp. 385, 394 (N.D. Ill. 1997).

IV. THE NATURE AND EXTENT OF THE SERVICES GARRET REQUIRES, ALONG WITH THE BURDEN ON THE DISTRICT, DEMONSTRATE THAT THE SERVICES HERE CONSTITUTE EXCLUDED MEDICAL SERVICES.

Instead of mechanically following a bright-line physician/nonphysician test, courts should consider several factors in determining whether the services are excluded medical services or included school health services. These factors include the nature and complexity of the care, whether the care is continuous or intermittent, whether existing school health personnel may provide the services, the cost of the services, and the potential repercussions if the services are not properly performed. These can be broadly

summarized as the nature and extent of the care and the burden on the district.

This individualized consideration of needed services for each child fits within the general mandate of the Act that programs for disabled children be developed on an individualized basis. The Act requires that there be individualized consideration of each student's disability, needs, goals and objectives, along with individualized determination of necessary educational, support and related services. 20 U.S.C. § 1401(a)(20). Throughout the Act, there is an emphasis upon recognizing each child's unique needs and necessary services, by using the expertise of the members of the individualized education program ("IEP") team.

The IEP team in Cedar Rapids determined that Garret required care that was beyond the scope of services which could be provided by the Cedar Rapids school nurses. EX 91 at 364-378; J.A. at 14-16. The team made a detailed listing of Garret's health care needs, of the specific interventions required, and of the frequency, and then determined that Garret needed "continuous care by an appropriately licensed practitioner" because Garret "requires continuous monitoring and assessment: highly technical and extensive nature of care, life-threatening prospect of inadequate care, mastery of numerous competencies." EX 91 at 374. All of the participating members of the team, including two school nurses and the school nurse manager, agreed with this assessment, with the exception of Garret's mother, Charlene F. J.A. at 14-16; EX 91 at 365; TR at 331-332, 338, 451-452.

The Iowa Board of Nursing also formally ruled that the care required by Garret could not be delegated to nonlicensed practitioners, such as a trained layperson, because of:

1. The complex nature of the student's care.

2. The number of activities that include the core of the nursing process and require specialized nursing knowledge, judgment and skill.

3. The school nurse/student ratio of 5.8/17,500 which severely limits the amount of training and supervision the registered nurse would be able to provide the nonlicensed personnel.

4. The absence of a school nurse in the building at all times. At times, the nurse may be 30 minutes away.

5. The potential risk of serious harm/injury to the student if he does not receive adequate care. [EX 125 at 456.]

Dr. Reinertson, Garret's personal physician, felt that a trained nonlicensed person could perform the services, as long as a licensed person was readily available by telephone. J.A. at 4-10. However, Dr. Reinertson also agreed that the requirements of the Iowa Board of Nursing are mandatory no matter what he believes. TR at 60.

All of the professionals, including Garret's doctor, agreed that Garret needed continuous care by a trained individual, not just a classroom teacher or by a school nurse responsible for an entire school of children. EX 91 at 364-388. As Dr. Reinertson wrote to the District: Garret should "never" be left unattended. "Someone should always be within hearing distance to respond to Garret's most basic needs and ventilator alarms." EX 91 at 388; TR at 408-09.

However, neither this case, nor any future case involving needed nursing services, necessarily depends on State licensure requirements for nurses. Whether a registered nurse with a bachelor's degree is required, or a registered nurse from a two-year program, or a licensed practical nurse, or a nonlicensed nurses aide, is irrelevant to a determination of whether the nursing services constitute excluded "medical services."

The courts which have previously considered whether one-on-one nursing services constitute medical services did not need to make distinctions based upon licensure. In *Detsel v. Board of Educ. of Auburn Enlarged City School Dist.*, 637 F. Supp. 1022 (N.D. N.Y. 1986), *aff'd*, 820 F.2d 587 (2d Cir.) (per curiam), *cert. denied*, 484 U.S. 981 (1987), the school physician testified that Melissa Detsel would require the services of either an LPN or a registered nurse. In *Fulginiti v. Roxbury Tp. Public Schools*, 921 F. Supp. 1320 (D.N.J. 1996), *aff'd without published opinion*, 116 F.3d 468 (3rd Cir. 1997), Carissa Fulginiti received the services of a licensed practical nurse under the federally funded Medical Assistance Program; the court found that even services provided by a lay person may qualify as excluded medical services. Similarly, the student in *Granite School Dist. v. Shannon M.*, 787 F. Supp. 1020 (D. Utah 1992), was receiving the services of a licensed practical nurse, as was the student in *Spencerport Central School Dist.*, 16 Educ. Handicap L. Rep. 1042 (NY Comm'r Educ. 1990). In *Glen Rock Board of Educ.*, 16 Educ. Handicap L. Rep. 1102 (NJ Admin. ALJ 1990), a state administrative law judge ruled that the services of a trained aide for a single student were excluded medical services. In *Neely v. Rutherford County School*, 68 F.3d 965 (6th Cir. 1995), *cert. denied*, 517 U.S. 1134 (1996), there was some argument over the licensure requirement for Samantha Neely's care, but it was agreed that Samantha required at most a licensed practical nurse; the Sixth Circuit Court of Appeals held that even if services could be provided by a nonlicensed nursing assistant, they will still be excluded medical services. *Id.* at 971.

While the administrative law judge placed primary reliance on a bright-line, physician/nonphysician distinction, he also indicated that if he were to use such a balancing test, the continuous nursing services required by Garret would be related services. Pet. App. 52a-53a. He

based this apparently upon a finding that students in some of the other cases were more "fragile." He stated:

> [w]hile Garret needs much in the way of the same health care services as the students in those cases, his general good health, ability to communicate his needs, intelligence and other factors weigh heavily in distinguishing him from the students in these rulings. Clearly, the students in *Detsel, Bevin H.*, and *Shannon M.* were more fragile in the true sense of the word. While testimony differed at the hearing as to the proper level of licensure needed by the person to care for Garret at school, it was obvious that in most situations a conscientious lay person could be, and has been in the past, trained to provide the necessary care. Unless Garret's situation changes in the future, such as a reoccurrence of antonemic (sic) hyperreflexia, which may need assessment of a professional nature on a reoccurring basis, there appears no practical need for Garret to be attended by even a licensed health care professional. [Pet. App. 52a.]

The administrative law judge then concluded that the additional $20,000-30,000 cost was not unduly burdensome. Pet. App. 53a.

However, this was not a true balancing test, but merely a comparison of Garret's case to some prior reported decisions. The administrative law judge failed to acknowledge that in all of these cases the need for one-on-one private duty type nursing was a determining factor. In all of these cases there was a need for an individual to exercise professional judgment in determining whether the student was in distress; in all of these cases there was the possibility of death for the student if the student was not properly administered health care services; and in all of these cases there were significant costs to the districts in hiring a trained person because existing school nursing services would be inadequate to provide for the student who needed continuous care. These are exactly the same features of

Garret's situation. Even Garret's mother and physician indicated Garret had and could continue to suffer episodes of autonomic hyperreflexia, which if not immediately alleviated, could lead to his death, necessitating continuous monitoring. EX 50 at 174-175, 179; EX 91 at 386; TR at 44-45, 52, 53.

This pragmatic test is no more difficult to apply than many other tests utilized to evaluate whether services are required for disabled children. In fact, if this Court determines that continuous, one-on-one nursing or other trained attendant services which cannot be performed by a school nurse with other responsibilities are inherently burdensome to a district, the test becomes merely a factual finding. Such findings are routinely made by IEP teams.

Schools should not be taken out of traditional roles they have played without a clear legislative mandate. As Congress previously found, schools should not be "asked to do what they are not intended to do." H. Rep. No. 101-544 at 11 (1990). This Court in *Rowley* extensively reviewed the statutory language and legislative history of the Act and concluded that Congress did not intend a broad interpretation that would impose "upon the States a burden of unspecified proportions and weight." 458 U.S. at 190, n. 11. This Court ruled that requirements for local districts to assume large or onerous financial responsibilities under the Act must be spelled out unambiguously:

> "[L]egislation enacted pursuant to the spending power is much in the nature of a contract: in return for federal funds, the States agree to comply with federally imposed conditions. The legitimacy of Congress' power to legislate under the spending power thus rests on whether the State voluntarily and knowingly accepts the terms of the 'contract.' . . . Accordingly, if Congress intends to impose a condition on the grant of federal moneys, it must do so unambiguously." [458 U.S. at 204, n. 26, citing *Pennhurst State School v. Halderman*, 451 U.S. 1, 17, 101 S. Ct. 1531, 1539-40, 67 L. Ed. 2d 694 (1981).]

CONCLUSION

The decision of the Eighth Circuit Court of Appeals should be reversed.

Respectfully submitted,

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